



data on these drugs anywhere, followed 753 American adults with obesity who started a GLP-1. Daily steps fell from about 5,047 to 4,487 and moderate-to-vigorous activity from 28 minutes a day to 22. Losing weight did not make anyone move more. That cohort averaged 52 years of age, so I am not putting those numbers on a professional athlete. But train slightly less inside a long deficit and you lose lean tissue from both ends.

Muscle is not cosmetic in cricket. It is your fifth over in a spell, your throw from the deep in the 48th, your ability to decelerate on a damp outfield without tearing a hamstring. Bone matters too. One randomised trial in adults at higher fracture risk found small reductions in hip and spine bone density on semaglutide. Fast bowlers already carry a lumbar stress-fracture risk, and that is not a group I would put into a long pharmacological energy deficit.

One argument here is not about vanity, and it deserves a hearing. Endurance capacity is usually reported relative to body weight.

An athlete of 70 kg with an absolute VO2max of 4.2 litres a minute reads 60 ml/kg/min. At 63 kg with the same 4.2 litres, he reads 66.7, an apparent 11 per cent gain with no improvement to the aerobic machinery. For a runner, a cyclist or anyone making a weight category, that is not trivial, and I can see a physician using it carefully with an athlete carrying surplus weight. That is my reasoning, not a published finding. But note what it is: a weight lever, not a fitness lever. If part of the loss is muscle, the absolute figure falls while the ratio flatters

Lean trap: Weight-loss medications make sense for those living with obesity, with fat to spare — not young athletes already carrying low body-fat percentages. SPECIAL ARRANGEMENT

you. And cricket is not a power-to-weight sport. Nobody here is chasing a VO2max number.

Then there is match day. A drug that slows gastric emptying and blunts appetite works against everything we try to do with an athlete over five hours in Indian heat, and nausea and vomiting are known side effects. Add a player who cannot stomach fluid or carbohydrate at the drinks break in a city like Chennai in April and you have cramping, poor decisions late on, and heat illness.

Two more things worry me. Weight regain after stopping is the norm, and the STEP 1 follow-up found about two-thirds of the loss came back within a year. A career spent cycling up and down like that is not conditioning; it is churn. And appearance-driven weight loss in a 19-year-old is how disordered eating begins. That is not a weakness on his part. It is what happens when we tell young men their abs are part of the job.

If any of this is happening, the fix is not only at the pharmacy. Boards and academies set the demand.

Body-composition targets should come from performance requirements, not broadcast angles or sponsor shoots. Track muscle mass instead of shouting about the weighing scale, and route any legitimate prescription through the team doctor, in writing, with a medical reason.

There is no injection that gives you four hours of skill work. A six-pack is a by-product of good training and good food. It was never supposed to be the target.